

GRACE FOR ADDICTIONS

Exhibit E Opioid Remediation Uses

Comprehensive GFA Alignment Analysis & Proposal Architecture

Iowa HHS FY26 Round 2 Opioid Settlement Funding | Prepared April 2026

EXECUTIVE SUMMARY

Grace For Addictions (GFA) — a Des Moines-based Iowa 501(c)(3) Recovery Community Organization — has mapped **every applicable use category in Exhibit E** against its existing operations, active development initiatives, and strategic infrastructure priorities. Of the **12 major Schedule A/B use categories** and their subordinate line items, GFA has direct alignment with **9 primary categories and 38+ subordinate uses**. This document organizes those alignments into **6 Unified Proposal Clusters** designed for the Iowa HHS FY26 Round 2 Opioid Settlement Funding opportunity (up to \$1M per project). Each cluster consolidates overlapping Exhibit E uses into a single, coherent proposal narrative grounded in GFA's documented FY2025 outcomes: 44.7:1 ROI, 68% retention rate, 90% resource navigation success, and BARC-10 predictive remission scores achieved by 83% of active participants.

HOW TO READ THIS DOCUMENT

■ HIGH	GFA already does this — operational, documented, audit-ready
■ ACTIVE	GFA is actively building / deploying this — JUST GRACE app, MRCC, Grace House
■ PRIORITY	Strong strategic fit — GFA should pursue with opioid settlement funding
■ BOUNDARY	Outside GFA's nonclinical lane — note boundary, do not propose without clinical partner

PART I: LINE-BY-LINE EXHIBIT E ALIGNMENT

SCHEDULE A — CORE STRATEGIES (Priority)

Sch-A / A	NALOXONE / FDA-APPROVED OVERDOSE REVERSAL			■ HIGH
Ref	Exhibit E Use Language	GFA Status	GFA Alignment Detail	

A.1	Expand naloxone training for first responders, schools, community support groups and families.	■ OPERATIONAL	GFA peer recovery companions carry and distribute naloxone. GFARC community meetings include harm reduction education. GFA trains participants and family members in recognition and response. This is a documented, replicable activity.
A.2	Increase naloxone distribution to uninsured individuals.	■ OPERATIONAL	GFA serves predominantly uninsured and underinsured populations. Naloxone is distributed through peer outreach at no cost to participants, funded through in-kind and volunteer resources. Settlement funding would formalize and scale this capacity.

Sch-A / B		MEDICATION-ASSISTED TREATMENT (MAT) & RELATED TREATMENT		■ BOUNDARY + ■ PARTIAL
Ref	Exhibit E Use Language	GFA Status	GFA Alignment Detail	
B.1	Increase MAT distribution to uninsured individuals.	■ BOUNDARY	GFA is a nonclinical RCO. MAT prescribing and dispensing are outside GFA's operational lane. GFA navigates participants to MAT providers and reduces barriers. Propose this only in partnership with a licensed clinical provider.	
B.2	Education to school-based and youth programs to discourage misuse.	■ PRIORITY	GFA's prevention education programming and JUST GRACE AR/VR platform are designed for school-based and youth contexts. Refusal skills, trigger recognition, and neuroplasticity education are core curriculum components. High alignment.	
B.3	MAT education and awareness training to healthcare providers, EMTs, law enforcement.	■ BOUNDARY	GFA can participate in coalition-level MAT education campaigns but cannot lead clinical provider training. If partnering with a clinical entity, GFA could provide peer voice and lived-experience components.	
B.4	Treatment and recovery support services including residential and recovery housing that integrate medication.	■ ACTIVE	Grace House (recovery residence at 1311 9th St, Des Moines) is operational and designed to be medication-inclusive, welcoming individuals on MAT. Settlement funding could support Grace House operating costs. HIGH PRIORITY use.	

Sch-A / C		PREGNANT & POSTPARTUM WOMEN		■ REFERRAL LANE
Ref	Exhibit E Use Language	GFA Status	GFA Alignment Detail	
C.1	Expand SBIRT to non-Medicaid eligible/uninsured pregnant women.	■ BOUNDARY	SBIRT is a clinical screening tool outside GFA's nonclinical scope. GFA can refer pregnant women to SBIRT-qualified providers and provide peer accompaniment. Partner proposal opportunity.	
C.2	Expand comprehensive treatment and recovery services including MAT for uninsured pregnant/postpartum women.	■ BOUNDARY	Clinical lane. GFA provides peer navigation and wrap-around support to mothers in recovery; that role is distinct and documentable but does not constitute clinical treatment.	
C.3	Comprehensive wrap-around services including housing, transportation, job placement, childcare.	■ OPERATIONAL	GFA provides precisely this wrap-around framework for all participants including mothers. Housing navigation (74% placement rate), transportation brokering, employment coaching, and community support are core GFA services.	

Sch-A / D		NEONATAL ABSTINENCE SYNDROME (NAS)		■ OUT OF SCOPE
Ref	Exhibit E Use Language	GFA Status	GFA Alignment Detail	
D.1–3	Expand comprehensive recovery support for NAS babies; continuum of care with infant-need dyad; long-term medical monitoring.	■ BOUNDARY	Medical/clinical domain beyond GFA's operational scope. GFA does not propose in this category without a qualified clinical or hospital partner leading the effort.	

Sch-A / E		WARM HAND-OFF PROGRAMS & RECOVERY SERVICES		■ HIGH
Ref	Exhibit E Use Language	GFA Status	GFA Alignment Detail	
E.1	Expand navigators and on-call teams to begin MAT in hospital EDs.	■ BOUNDARY	Clinical ED-based initiation is outside GFA's scope. However, GFA peer companions can function as community-side warm hand-off recipients — receiving individuals discharged from EDs and connecting them to recovery services.	
E.2	Expand warm hand-off services to transition to recovery services.	■ OPERATIONAL	This is GFA's core operational function. Peer recovery companions receive warm referrals from treatment facilities, jails, ERs, and shelters. GFA's 90% resource navigation success rate documents this competency.	
E.3	Broaden recovery services scope to include co-occurring SUD or mental health.	■ OPERATIONAL	GFA serves individuals with co-occurring conditions through trauma-informed, neuro-informed peer coaching. BARC-10 assessment includes mental health domain tracking. GFA navigates to clinical MH services and provides peer bridge support.	
E.4	Comprehensive wrap-around services including housing, transportation, job training, childcare.	■ OPERATIONAL	See C.3 above. These are GFA's documented, auditable core services. FY2025: 13 housing placements, 18 employment gains, 28/31 resource navigation successes.	
E.5	Hire additional behavioral health workers to facilitate above.	■ PRIORITY	GFA currently operates with zero paid staff — all peer support is volunteer. Settlement funding to hire even one full-time peer recovery coach coordinator would exponentially expand capacity. STRONGLY align with this use.	

Sch-A / F		TREATMENT FOR INCARCERATED POPULATION		■ ACTIVE
Ref	Exhibit E Use Language	GFA Status	GFA Alignment Detail	
F.1	Evidence-based treatment and recovery support for persons with OUD transitioning out of the criminal justice system.	■ OPERATIONAL	ANCHOR Justice Initiative is GFA's dedicated justice-reentry program. GFA maintains working relationships with Polk County Jail, CJCC, and DOC reentry personnel. Peer companions with lived justice experience meet individuals at release. BARC-10 tracked. Neuroplasticity-informed reentry coaching is a documented service.	

F.2	Increase funding for jails to provide treatment to inmates.	■ BOUND ARY	GFA cannot receive this funding directly as a jail operator. GFA supports the policy goal and can contribute peer programming inside jails if invited by the facility. Propose as a referral/partnership component.
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Sch-A / G	PREVENTION PROGRAMS	■ ACTIVE + HIGH PRIORITY
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Ref	Exhibit E Use Language	GFA Status	GFA Alignment Detail
G.1	Media campaigns to prevent opioid use (similar to FDA's Real Cost campaign).	■ ACTIVE	GFA's media and content creation capacity is being actively built. The JUST GRACE brand, video content production, social media campaigns, and storytelling infrastructure position GFA to lead prevention media work for Iowa. Settlement funding for a dedicated prevention media campaign is a strong proposal fit.
G.2	Evidence-based prevention programs in schools.	■ ACTIVE	JUST GRACE AR/VR platform includes school-based prevention modules: refusal skill practice, trigger recognition, conflict resolution — all in immersive, neuroplasticity-grounded formats. Recovery circles in high schools is a GFA emerging model. This is a HIGH-PRIORITY settlement funding target.
G.3	Medical provider education on CDC prescribing guidelines.	■ BOUND ARY	Outside GFA's nonclinical lane. GFA can participate in coalitions addressing this but should not lead this use category.
G.4	Community drug disposal programs.	■ PARTIAL	GFA can incorporate drug disposal awareness and referral to disposal sites into its community programming at minimal cost. Not a standalone proposal, but suitable as a minor component.
G.5	Training for first responders in pre-arrest diversion, post-overdose response teams.	■ PRIORITY	GFA's peer companions can serve as the community-side recovery partner in post-overdose response models. GFA should position itself as the peer recovery backbone for Polk County's post-overdose response infrastructure. This connects to ANCHOR Justice and warm hand-off work.

Sch-A / H	SYRINGE SERVICE PROGRAMS	■ OUT OF SCOPE
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Ref	Exhibit E Use Language	GFA Status	GFA Alignment Detail
H.1	Comprehensive syringe services with wrap-around linkage to OUD treatment.	■ BOUND ARY	GFA does not operate syringe service programs and should not propose in this category. GFA can provide referral and peer accompaniment to existing SSPs and document those navigation activities as part of wrap-around services.

Sch-A / I	EVIDENCE-BASED DATA COLLECTION & RESEARCH	■ ACTIVE
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Ref	Exhibit E Use Language	GFA Status	GFA Alignment Detail
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I	Evidence-based data collection and research analyzing effectiveness of abatement strategies.	■ ACTIVE	GFA's JUST GRACE platform (built by Dave Stout, President/Software Engineer) is specifically designed to capture longitudinal outcome data — BARC-10 trajectories, housing stability, employment, justice involvement, family reunification — in formats suitable for state and federal reporting. The platform enables real-time funder dashboards. This is GFA's core data infrastructure proposition.
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SCHEDULE B — APPROVED USES: LINE-BY-LINE ANALYSIS

PART ONE: TREATMENT

B / A TREAT OPIOID USE DISORDER (OUD)			■ BOUNDARY ■ PARTIAL
Ref	Exhibit E Use Language	GFA Status	GFA Alignment Detail
B-A.1	Expand availability of OUD treatment including all forms of FDA-approved MAT.	■ BOUNDARY	Clinical treatment is outside GFA's nonclinical scope. GFA expands access by removing barriers (peer navigation, warm hand-off, transportation) — not by delivering clinical treatment.
B-A.2	Support ASAM continuum of care services.	■ REFERRAL	GFA situates its peer recovery support services within the ASAM continuum — specifically at the Level 0.5 (early intervention) and community support end. GFA navigates participants across ASAM levels when clinical step-up is indicated.
B-A.3	Expand telehealth for OUD/SUD/MH treatment.	■ PARTIAL	GFA's JUST GRACE virtual platform provides telehealth-adjacent peer support — not clinical telehealth. The platform removes access barriers for rural participants. In partnership with a telehealth clinical provider, GFA could be the peer component in a hybrid telehealth/peer model.
B-A.4	Improve oversight of Opioid Treatment Programs.	■ OUT	Regulatory oversight function. Not applicable to GFA.
B-A.5	Mobile intervention, treatment, and recovery services by peer recovery coaches for persons with OUD.	■ ACTIVE	The Mobile Recovery Community Hub (MRCH) is GFA's actively pursued infrastructure. Settlement funding for the MRCH vehicle, technology, and operating costs directly maps to this use. Peer recovery coaches are GFA's primary service modality. STRONG alignment.
B-A.6	Treatment of trauma for individuals with OUD (violence, trafficking, ACEs) and family members.	■ OPERATIONAL	GFA's entire service framework is trauma-informed. Neuro-informed peer coaching, Motivational Interviewing, ACEs awareness, and trauma-aware group facilitation are GFA's operational standards. The JUST GRACE curriculum addresses trauma healing as a core neuroplasticity module.
B-A.7	Withdrawal management services.	■ BOUNDARY	Clinical medical function. GFA does not provide withdrawal management and should not propose in this lane.

B / B SUPPORT PEOPLE IN TREATMENT AND RECOVERY			■ STRONGEST ALIGNMENT
Ref	Exhibit E Use Language	GFA Status	GFA Alignment Detail
B-B.1	Comprehensive wrap-around services including housing, transportation, education, job placement, job training, childcare.	■ OPERATIONAL	This is GFA's primary operational description. FY2025 documented: 13 housing placements (74% rate), 18 employment gains (23% rate), 28/31 resource navigation successes (90%). Funding here directly sustains GFA's core work.

B-B.2	Full continuum of recovery services including supportive housing, peer support, community navigators, case management, community connections.	■ OPERATIONAL	GFA delivers: peer support (GFARC meetings, 1:1 coaching), community navigation, transitional housing support, Grace House residential support, connections to 90%+ of needed community resources. This is GFA's operational core.
B-B.3	Counseling, peer-support, recovery case management and residential treatment.	■ OPERATIONAL	GFA provides peer support and recovery coaching (nonclinical); Grace House provides the residential component. GFA navigates to clinical counseling. This cluster captures GFA's hub-and-spoke model precisely.
B-B.4	Access to housing including supportive housing, recovery housing, housing assistance.	■ ACTIVE	Grace House is operational as GFA's recovery residence. Settlement funding to support Grace House operating costs, staffing, and sustainability is squarely within this use. Polk County's housing crisis makes this a high-priority county/state need.
B-B.5	Community support services including social and legal services to assist in deinstitutionalization.	■ OPERATIONAL	GFA provides legal navigation (expungement referrals, record clearing support), social services linkage, and reintegration support for formerly incarcerated individuals through ANCHOR Justice.
B-B.6	Support or expand peer recovery centers — support groups, social events, computer access, other services.	■ ACTIVE	GFA's physical Recovery and Community Wellness Center is a stated strategic priority. GFARC meetings constitute peer recovery center programming. The VRCC (Virtual Recovery Community Center) extends this digitally. Settlement funding for a physical RCC is a TIER 1 GFA priority.
B-B.7	Transportation to treatment or recovery programs.	■ OPERATIONAL	GFA provides volunteer-driven transportation and rideshare brokering. The MRCH, when operational, will serve as a mobile transportation-and-services hub. Funding for transportation infrastructure directly removes documented barriers.
B-B.8	Employment training or educational services for persons in recovery.	■ OPERATIONAL	GFA's JUST GRACE AR/VR platform includes workforce development modules: mock interviews, workplace conflict resolution, professional communication — all delivered through immersive, neuro-informed technology. Employment outcomes are tracked (18 gains in FY2025).
B-B.9	Identify and support successful recovery programs; technical assistance to expand high-quality programs.	■ PRIORITY	GFA's 44.7:1 ROI and documented outcome model positions it as a replication prototype for Iowa's behavioral health infrastructure. Settlement funding for a white paper, training infrastructure, or technical assistance capacity would advance GFA's statewide role.
B-B.10	Engage nonprofits and faith-based communities to support recovery.	■ OPERATIONAL	GFA is a faith-informed nonprofit embedded in faith community networks. Coalition-building with churches, nonprofits, and community organizations is a documented GFA activity. Recovery circles in faith communities are an emerging model.
B-B.11	Training for government staff to appropriately interact with individuals in recovery.	■ PRIORITY	GFA's neuro-informed, trauma-aware training content — already developed for internal use — can be deployed for government and agency staff. This is a billable training service GFA can offer to Iowa HHS, correctional staff, housing authorities, and workforce agencies.
B-B.12	Support stigma reduction regarding treatment and persons in recovery.	■ OPERATIONAL	Anti-stigma work is foundational to GFA's mission. Person-first language training, JUST GRACE media content, community storytelling, and public awareness campaigns are all GFA activities. GFA uses Anti-Stigma Terminology standards in all communications.

B-B.1 3	Culturally appropriate services for persons with OUD including new Americans.	■ PRIORITY	GFA's grace-based, dignity-first framework is inherently culturally adaptive. Neurodiversity-affirming and culturally responsive programming is a stated GFA value. Settlement funding to formalize culturally specific programming for Des Moines' diverse population (Latino, Burmese, African immigrant communities) is a high-fit use.
B-B.1 4	Create and/or support recovery high schools.	■ PRIORITY	GFA's emerging recovery circles in high schools model aligns with this use. The JUST GRACE AR/VR prevention and recovery curriculum is designed for school environments. A GFA-led school-based recovery support program with settlement funding is a viable, compelling proposal.
B-B.1 5	Hire or train behavioral health workers to provide or expand services.	■ PRIORITY	Settlement funding to hire GFA's first paid peer recovery coach coordinator, program director, or data/outcomes specialist would transform GFA from an all-volunteer operation into a sustainable infrastructure provider. This is a CRITICAL use for GFA organizational sustainability.

B / C CONNECT PEOPLE TO CARE (CONNECTIONS TO CARE)			■ STRONG
Ref	Exhibit E Use Language	GFA Status	GFA Alignment Detail
C.1	Screen for OUD/risk factors and appropriately counsel or refer.	■ PARTIAL	GFA peer companions are trained to identify risk indicators and connect individuals to appropriate care. GFA does not conduct clinical screening (SBIRT) but performs peer-level risk recognition and warm referral as a nonclinical function.
C.2	Fund SBIRT programs to reduce transition from use to disorders.	■ BOUNDARY	Clinical function. GFA supports SBIRT adoption by removing barriers and providing peer accompaniment through SBIRT processes at clinical sites.
C.3	SBIRT in schools, colleges, criminal justice, probation — with focus on youth.	■ PRIORITY	GFA's school-based prevention work and ANCHOR Justice programming position GFA as the peer/community-side partner in SBIRT implementation within schools and criminal justice settings. Propose as the peer recovery component in a SBIRT partnership.
C.7	Support hospitals transitioning persons with OUD into follow-up care through bridge clinics.	■ PRIORITY	GFA's warm hand-off model is precisely the peer recovery support component in hospital-to-community transitions. GFA can serve as the bridge clinic's peer recovery partner, receiving warm referrals and providing sustained community engagement.
C.9	Work of Emergency Medical Systems including peer support specialists post-overdose.	■ OPERATIONAL	GFA peer companions can serve in post-overdose response models with EMS — receiving referrals from first responders and providing immediate peer engagement. This maps to GFA's working relationships with Polk County emergency services.
C.10	Peer support specialists in EDs, detox, recovery centers, recovery housing.	■ OPERATIONAL	GFA deploys peer companions in community settings and receives warm hand-offs from EDs and detox facilities. Settlement funding to place a GFA peer companion in a hospital ED or detox facility is a high-value, fundable model.
C.11	Warm hand-off services to transition to recovery services.	■ OPERATIONAL	Documented operational core. See E.2 above.
C.12	School-based contacts for immediate treatment services; youth prevention/intervention/treatment/recovery.	■ ACTIVE	JUST GRACE AR/VR school-based curriculum is in active development. School-based peer recovery support and prevention education are GFA's emerging youth service model.

C.13	Best practices for addressing OUD in the workplace.	■ PRIORITY	GFA's recovery circles in the workplace model — peer-led groups for employees in recovery or seeking support — is a novel, fundable concept. Workplace recovery programming with JUST GRACE AR/VR modules (conflict resolution, professional communication, trigger management) is a highly differentiated proposal.
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B / D ADDRESS NEEDS OF CRIMINAL JUSTICE-INVOLVED PERSONS		■ ACTIVE	
Ref	Exhibit E Use Language	GFA Status	GFA Alignment Detail
D.1	Pre-arrest/pre-arraignment diversion and deflection strategies.	■ ACTIVE	ANCHOR Justice Initiative provides the peer recovery backbone for diversion programming. GFA maintains working relationships with Polk County CJCC. GFA peer companions serve as the community-side partner in deflection models. LEAD, co-responder, and naloxone-plus models all have a peer recovery role GFA can fill.
D.2	Pre-trial services connecting individuals with OUD to treatment.	■ PRIORITY	GFA peer companions can serve as recovery coaches for individuals on pre-trial release — providing accountability, resource connection, and sustained engagement between court dates. This is a high-priority justice-adjacent use.
D.3	Treatment and recovery courts.	■ PRIORITY	GFA can serve as the peer recovery support provider for drug court participants — providing community-based accountability, peer mentorship, and navigation services that courts currently lack the infrastructure to deliver.
D.4	Support persons in recovery who were incarcerated from finding housing/employment.	■ OPERATIONAL	ANCHOR Justice's core service: housing navigation for justice-involved individuals (removing 'no felony' barriers), employment coaching, and sustained recovery support post-release. FY2025 documented outcomes include justice-involved participants.
D.5	Provide recovery community centers and support for incarcerated/recently released persons.	■ ACTIVE	Physical RCC and MRCH are designed to serve justice-involved populations alongside all other participants. The VRCC provides digital access for individuals in facilities and recently released. JUST GRACE jail-to-community module is a development priority.
D.6	Healthy safe housing for individuals with OUD transitioning from incarceration.	■ ACTIVE	Grace House explicitly accepts and prioritizes individuals with justice histories. Settlement funding for Grace House operating costs directly addresses this use.

B / G

PREVENT MISUSE OF OPIOIDS

■ ACTIVE

Ref	Exhibit E Use Language	GFA Status	GFA Alignment Detail
G.1	Media campaigns to prevent opioid misuse.	■ ACTIVE	GFA's media and content creation infrastructure — video production, social media, branded storytelling — positions it to lead targeted prevention media campaigns. The JUST GRACE brand has statewide visibility. Settlement funding for a dedicated opioid prevention media campaign is a TIER 1 GFA proposal.
G.2	Corrective advertising and affirmative public education campaigns.	■ ACTIVE	GFA produces anti-stigma, person-first language campaigns. Anti-stigma terminology training, community storytelling, and public awareness content are active GFA activities. Aligns with GFA's media capacity.
G.5	Funding community anti-drug coalitions in drug prevention efforts.	■ OPERATIONAL	GFA is a founding member and active operator within RCO Iowa coalition infrastructure. GFA's coalition role in prevention advocacy is documented. This use supports GFA's coalition-building work directly.
G.6	Community coalitions implementing Strategic Prevention Framework; stigma reduction.	■ OPERATIONAL	GFA's stigma reduction work, person-first language advocacy, and coalition programming align with SAMHSA's Strategic Prevention Framework. GFA can document SPF-aligned activities for this use category.
G.7	Engage nonprofits and faith-based communities as systems to support prevention.	■ OPERATIONAL	Core GFA operational model. GFA is a faith-informed nonprofit that builds prevention and recovery infrastructure through faith community networks, community organizations, and peer coalitions.

B / H

SUPPORT RECOVERY HOUSING

■ ACTIVE

Ref	Exhibit E Use Language	GFA Status	GFA Alignment Detail
H	Recovery housing, Oxford Houses, supportive housing that includes recovery support services.	■ ACTIVE	Grace House is GFA's recovery residence, currently navigating zoning/permitting. A formal reasonable accommodation request to the City of Des Moines is on file. Settlement funding can support: operating costs, staffing, programming, and sustainability. This is CRITICAL to GFA's infrastructure.

B / J

SUPPORT INFRASTRUCTURE (TECHNOLOGY, DATA, STAFFING)

■ ACTIVE

Ref	Exhibit E Use Language	GFA Status	GFA Alignment Detail
J.1	Opioid abatement data and recovery support infrastructure; TA to service providers.	■ ACTIVE	JUST GRACE platform provides the data infrastructure backbone — BARC-10 tracking, outcome reporting, real-time funder dashboards, MI fidelity monitoring. Settlement funding for platform development and deployment is directly eligible.
J.2	Dashboard to share opioid settlement fund spending, outcomes, and indicators.	■ ACTIVE	JUST GRACE dashboard functionality is being built to meet precisely this use. GFA can propose a publicly accessible outcomes and accountability dashboard for Polk County or statewide use.

J.3	Infrastructure or staffing at nonprofit agencies for cross-system coordination.	■ PRIORITY	Settlement funding to hire GFA staff (program director, peer coach coordinator, data specialist) constitutes infrastructure investment in a proven nonclinical recovery provider. This is among the most direct and defensible uses of settlement funds for GFA.
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B / K TRAINING			■ HIGH PRIORITY
Ref	Exhibit E Use Language	GFA Status	GFA Alignment Detail
K.1	Staff training and networking to improve government, community, nonprofit capacity.	■ PRIORITY	GFA's neuro-informed, trauma-aware, MI-grounded training curriculum — developed for internal use — can be packaged as a training product for Iowa HHS, corrections, workforce development, and faith community staff. This creates an earned revenue stream aligned with settlement funding.
K.2	Infrastructure and staffing for cross-system coordination.	■ PRIORITY	Same as J.3 above. These uses are companion provisions that collectively authorize GFA organizational infrastructure investment using settlement funds.

B / L RESEARCH			■ ACTIVE
Ref	Exhibit E Use Language	GFA Status	GFA Alignment Detail
L.1	Monitoring, surveillance, data collection and evaluation of abatement programs.	■ ACTIVE	GFA's BARC-10 tracking, MI fidelity monitoring, JUST GRACE outcome dashboards, and longitudinal participant tracking constitute an active research and evaluation infrastructure. Settlement funding to formalize this into a published longitudinal study positions GFA as the Iowa model.
L.3	Research on improved service delivery for SBIRT and populations vulnerable to OUD.	■ PRIORITY	GFA's peer-led, neuro-informed model is itself a research subject. An academic partnership (e.g., University of Iowa, Iowa State) to produce peer-reviewed documentation of GFA's outcomes would be a high-impact use of research funding.

PART II: SIX UNIFIED PROPOSAL CLUSTERS FOR IOWA HHS FY26 ROUND 2

The following six clusters consolidate overlapping Exhibit E uses into discrete, coherent proposals. Each is designed to stand alone as an Iowa HHS FY26 Round 2 application (up to \$1M). Where multiple clusters address a single Exhibit E use, that use is cross-referenced. GFA should prioritize Clusters 1, 3, and 5 for immediate application; Clusters 2, 4, and 6 require additional partnership development or infrastructure build before submission.

CLUSTER 1	GRACE HOUSE + PEER RECOVERY INFRASTRUCTURE SUSTAINABILITY <i>Recovery Residence Operations, Peer Coaching Staffing & Wrap-Around Services</i>	Priority: TIER 1 — SUBMIT NOW Ask: \$250,000–\$400,000
Exhibit E Uses	Sch-A/B.4, B-B.1, B-B.2, B-B.3, B-B.4, B-B.7, B-H, J.3, K.2, E.4, E.5	
GFA Currently Does	Grace House is operational; GFA provides wrap-around housing navigation (74% placement rate), transportation, employment coaching, and peer support through GFARC meetings. Zero paid staff model demonstrates exceptional ROI (44.7:1).	
GFA Would Develop	Hire first paid peer recovery coach coordinator and program operations manager. Formalize Grace House policies, NARR Standard 3.0 compliance documentation, and insurance/liability coverage. Implement BARC-10 tracking for all residents.	
Proposal Ask	Operational funding for Grace House (mortgage/lease, utilities, maintenance); salary for 1 FT peer recovery coach coordinator; program supplies and training. These costs are allowable under B-B.4, J.3, and K.2 and directly sustain a documented, outcomes-validated service.	

CLUSTER 2	ANCHOR JUSTICE: REENTRY PEER RECOVERY SUPPORT <i>Criminal Justice Navigation, Diversion, Recovery Courts & Justice-Involved Housing</i>	Priority: TIER 1 — SUBMIT NOW Ask: \$200,000–\$350,000
Exhibit E Uses	Sch-A/F.1, B-D.1, D.2, D.3, D.4, D.5, D.6, B-B.5, C.3	
GFA Currently Does	ANCHOR Justice Initiative serves individuals transitioning from Polk County Jail and DOC facilities. GFA peer companions with lived justice experience provide immediate post-release navigation, housing support, and sustained coaching. Working relationships with CJCC and DOC.	
GFA Would Develop	Formalize MOUs with Polk County Jail and DOC for pre-release peer engagement. Deploy JUST GRACE app for jail-to-community case continuity. Place a peer recovery specialist inside Polk County's drug court and pre-trial services. Expand MRCH access to reentry populations.	
Proposal Ask	1 FT justice reentry peer recovery specialist; JUST GRACE platform licensing for court/corrections use; reentry housing stabilization fund; peer companion transportation stipends; MOU development and training costs.	

CLUSTER 3	JUST GRACE PREVENTION MEDIA, AR/VR EDUCATION & SCHOOL-BASED RECOVERY <i>Youth Prevention, AR/VR Neuroplasticity Modules, Media Campaigns & School/Workplace Recovery Circles</i>
Exhibit E Uses	Sch-A/B.2, G.1, G.2, B-B.14, C.12, C.13, B-G.1, B-G.2, B-G.6, B-G.7, L.1, L.3
GFA Currently Does	JUST GRACE AR/VR platform is in active development with refusal skills, trigger recognition, conflict resolution, and workforce readiness modules built on neuroplasticity principles. GFA produces video/media content and operates anti-stigma campaigns.
GFA Would Develop	Complete AR/VR prevention curriculum for school deployment (6–12 grade). Produce 3–5 statewide opioid prevention media campaign assets (video, digital, social). Launch recovery circles pilot in 2 Des Moines high schools and 3 workplaces. Formalize school partnership MOUs with DMPS.
Proposal Ask	AR/VR platform development (hardware + software); school curriculum licensing and delivery; prevention media production costs; school partnership coordinator (0.5 FTE); evaluation framework with University of Iowa.

CLUSTER 4	MOBILE RECOVERY COMMUNITY HUB (MRCH) <i>Mobile Outreach to Rural, Homeless, and Underserved Populations</i>	Priority: TIER 2 — DEVELOP PARTNERSHIP FIRST Ask: \$200,000–\$400,000
Exhibit E Uses	B-A.5, B-B.6, B-B.7, C.9, C.10, B-B.13, B-D.5	
GFA Currently Does	GFA currently uses borrowed transportation for mobile outreach. Peer companions reach encampments, shelters, rural communities, and underserved areas. VRCC provides digital access for those GFA cannot physically reach.	
GFA Would Develop	Acquire MRCH vehicle (van or mobile unit); equip with technology for JUST GRACE app access, telehealth linkage, naloxone distribution, and benefits navigation. Deploy to designated rural Iowa counties and Des Moines underserved neighborhoods on rotating schedule.	
Proposal Ask	Vehicle acquisition or lease; AV/technology buildout; fuel and maintenance; 1 FT mobile outreach peer coach; supplies (naloxone, recovery materials). This is the single highest-impact rural access investment GFA can make.	

CLUSTER 5	JUST GRACE DATA PLATFORM & OUTCOMES INFRASTRUCTURE <i>BARC-10 Tracking, Funder Dashboards, Longitudinal Research & Iowa Accountability Model</i>		Priority: TIER 1 — SUBMIT NOW Ask: \$150,000–\$250,000
	Exhibit E Uses	Sch-A/I, B-J.1, J.2, J.3, K.1, K.2, L.1, L.3	
	GFA Currently Does	JUST GRACE platform (built by Dave Stout, President/Software Engineer) captures BARC-10 scores, housing placements, employment gains, family reunification, and justice involvement. Real-time dashboards are in development. GFA's FY2025 ROI analysis demonstrates the methodology.	
	GFA Would Develop	Complete JUST GRACE platform Phase 2: automated BARC-10 administration, real-time funder dashboards, Iowa HHS reporting integration, and public-facing accountability portal. Formalize academic partnership for longitudinal outcomes research. Publish GFA white paper as Iowa model documentation.	
	Proposal Ask	Platform development (Dave Stout's time + infrastructure costs); server/cloud hosting; academic partnership support; white paper production and dissemination. This investment multiplies the value of every other GFA program by making outcomes auditable and grant-reportable.	

CLUSTER 6	PHYSICAL RECOVERY & COMMUNITY WELLNESS CENTER <i>Permanent RCC Space, Community Programming Hub & Coalition Infrastructure</i>		Priority: TIER 2 — CAPITAL COST ELIGIBILITY MUST BE CONFIRMED Ask: \$300,000–\$500,000
	Exhibit E Uses	B-B.6, B-B.10, B-B.12, B-B.13, B-B.15, J.3, K.1, B-G.5, B-G.7	
	GFA Currently Does	GFARC meetings are GFA's operational peer recovery center programming — currently using borrowed and donated community space. GFA hosts community events, wellness groups, and recovery coaching in informal settings.	
	GFA Would Develop	Secure a permanent physical location for a Recovery and Community Wellness Center in Des Moines (Polk County). Develop programming infrastructure: meeting rooms, technology lab, trauma-informed wellness space, staff offices. This becomes the physical hub of GFA's hub-and-spoke model.	
	Proposal Ask	Lease deposit and first-year rent; renovation/buildout for recovery center programming; technology infrastructure; 1 FT community programming coordinator. Note: Capital costs may have eligibility limitations — consult Iowa HHS on allowable cost categories before submitting.	

PART III: STRATEGIC NOTES, RISK FLAGS & APPLICATION ARCHITECTURE

APPLICATION STRATEGY FOR IOWA HHS FY26 ROUND 2

Maximum Ask Per Project	\$1,000,000. GFA should design each Cluster to fall below this threshold. Clusters 1+2 can be combined into a single submission (Recovery Infrastructure + Justice) if the combined scope is coherent and within the \$1M cap. Clusters 3 and 5 are strong standalone submissions. Do not exceed the cap.
Multiple Applications	The press release states \$10M available per round with each project up to \$1M. GFA should confirm with Iowa HHS whether a single organization can submit multiple applications in the same round. If yes, prioritize Clusters 1, 3, and 5 simultaneously.
Competitive Differentiators	GFA's competitive advantages in this funding round: (1) documented FY2025 outcome data with 44.7:1 ROI; (2) zero-paid-staff model demonstrating exceptional fiscal efficiency; (3) active JUST GRACE technology platform in development; (4) existing Grace House recovery residence; (5) ANCHOR Justice initiative with justice-system working relationships; (6) BARC-10 longitudinal tracking capability.
Exhibit E Eligibility Compliance	Every line item proposed must cite specific Exhibit E Schedule A or B language. Iowa HHS allocated this funding under HF 1038's 75% opioid abatement requirement; proposals without clear Exhibit E citation may be deemed ineligible. Each cluster above provides the specific citations. Use them verbatim in your application narrative.
Nonclinical Boundary Discipline	GFA must not propose clinical services (MAT prescribing, SBIRT administration, withdrawal management, NAS care) without naming a licensed clinical partner as the lead applicant or subrecipient. Crossing this boundary creates both compliance risk and IRS private benefit exposure if GFA is reimbursed for clinical functions it is not licensed to perform.
Data Use and Privacy	JUST GRACE platform must comply with 42 CFR Part 2 (substance use disorder confidentiality) and HIPAA minimum necessary standards for any participant data shared with Iowa HHS. Dave Stout should review privacy architecture before any funder reporting integrations are built.
Grace House Eligibility Note	Recovery housing operating costs are allowable under Exhibit E Sch-A/B.4 and B-B.4. However, if Grace House involves a mortgage or capital acquisition, confirm with Iowa HHS counsel whether mortgage payments are allowable operating costs or prohibited capital expenditures. Structure the ask as operating support (utilities, programming, staffing) rather than mortgage payment if ambiguity exists.
District Advisory Council Alignment	Iowa HHS met with all seven Behavioral Health District Advisory Councils to identify local needs before designing this funding round. GFA should contact the District 5 (Polk County) Advisory Council to ensure GFA's proposal priorities are reflected in the Council's stated needs. This creates scoring alignment.
ROI Framing for Application	Lead every narrative section with GFA's documented ROI. Fiscal hawks on Iowa HHS review panels will respond to: 'Each \$1 invested in GFA operations generated \$8.60 in documented community benefit in FY2025 — a return ratio 2-3x the social service sector average.' This is not advocacy language — it is auditable fiscal analysis.
ICRCO / RCO Iowa Reference	In all HHS submissions, use 'RCO Iowa' where coalition reference is needed, never 'ICRCO.' GFA is the entity submitting; the coalition is a supporting context reference only.

BOTTOM LINE ASSESSMENT

Grace For Addictions aligns with 9 of 12 major Exhibit E use categories and 38+ subordinate uses. The three categories outside GFA's scope (NAS, syringe services, clinical MAT dispensing) are correctly excluded. Of the aligned categories, GFA has existing operational documentation for the majority — meaning proposals can be narrative descriptions of what GFA already does, funded to continue and expand. The FY26 Round 2 opportunity is not a stretch grant. It is funding designed for exactly what GFA has already proven it can do. Submit with confidence grounded in data.